

Faculty

Student:

DOORWAY INFORMATION / CHART

Patient Name	Carol Meyers
Age/Date of Birth	55; 12/21/1966
Gender	Woman
Sex Assigned at Birth	Female
Clinical Setting	Outpatient Center / Primary Care Clinic Same Day Appointment
Chief Concern	Headache

Vital Signs	
Weight	176 lbs (79.8 kg)
Height	5' 4" (163 cm)
BMI	30.04
BP	145/95 mm Hg
Pulse	101/min
Respirations	19/min
Pulse oximetry	95% on RA
Temp	101.7 DegF (38.7 DegC)
Pain	10/10

Clinical Reasoning/Presentation/Treatment Plan:

This is a _____ with a PMH of _____, presenting with a CC of _____ x (Duration) _____.

Summary sentence: My working Diagnosis is: _____, supported by:

1. _____
2. _____
3. _____
4. _____ (need at least 4 – Prefer from both HX and PE)

Other possible diagnoses that are less likely (need at least one reason)

- | | |
|----------|---------------|
| 1. _____ | Reason: _____ |
| 2. _____ | Reason: _____ |
| 3. _____ | Reason: _____ |

I would like to implement the following treatment:

Medications:

Diagnostic Studies:

- Labs:
- Imaging:
- Other Studies:

OMT:

Education:

Follow Up:

- With PCP:
- Referral/Consultations:

NOTE: If a history question or physical exam finding is not listed here, proceed as if it is normal.

	Name of patient	Carol Meyers
	Clinical Setting	Outpatient Center / Primary Care Clinic
	Introductions How can I address you?	Carol is fine.
	HPI (OLDCARTS)	
	Chief Concern What brings you in today? <i>Empathy opportunity</i>	I have a horrible headache.
	Onset When did you first notice the pain? When did the headache start?	It started about 2 hours ago.
	Precipitating events Did anything in particular happen around that time? Did you injure your head? <i>Empathy opportunity</i>	I was out walking the dog and fell. I do not remember if I fell because of the headache or if the headache came because I hit my head in the fall – it was all so sudden.
	Previous episodes Have you had headaches like this before?	Nothing like this. I did have a pretty bad headache about 2 weeks ago, but it went away on its own.
	To clarify, this headache feels different from the headache from 2 weeks ago?	Yes, this is the worst headache of my life!
	Location Where is the pain?	My entire head hurts...
	Radiation Is the pain only in those places, or does the pain travel to other places?	It hurts all the way down to my neck.
	Progression/constant/intermittent Is it getting better, worse, or about the same? Is it constant or does it come and go?	The headache has just been horrible the entire time.
	Quality How would you describe the pain? Sharp, dull, throbbing, burning?	I cannot describe it. It is just incredibly painful.
	Aggravating factors Anything that makes the pain worse?	It gets even worse when I move. Bright lights and loud noises also make it worse.
	Relieving factors Anything that makes the pain better? <i>Empathy opportunity</i>	If I do not move. Can I please lie down? <i>Help the patient lie down so that they can be more comfortable.</i>
	Intensity (on a scale from 0-10)	A 10.

	On a scale from 0 to 10, with 0 being no pain and 10 being the worst pain you've ever experienced, what number would you rate it?	
	Associated Symptoms	Remember to ask about 1 symptom at a time.
	Any nausea?	I am sick to my stomach.
	Vomiting?	I had to throw up once on my way here.
	Fever?	I was sick with a fever last week, but it seemed like it had gotten better over the past few days.
	Tell me more about what was going on last week with feeling sick with a fever.	I think I had a sinus infection last week, when I also felt like I was running a fever. It had just gotten better though.
	Did you check your temperature? <i>Empathy opportunity ("No worries! I ask because my medical assistant checked your temperature and it's 101.7 DegF.")</i>	I did not, sorry.
	Chills?	No.
	Sick contacts Have you been around anyone who's sick?	No.
	Racing of the heart (palpitations)	Yes. I feel it right now.
	Rash?	No.
	Chest pain?	No.
	Shortness of breath?	No.
	Any dizziness, feeling as if your surroundings are spinning?	No, but I feel a little lightheaded right now.
	Numbness?	No.
	Tingling?	No.
	Weakness Any difficulty with moving your arms or legs?	No.
	Blurred vision?	No.
	Did anyone witness you having a seizure?	No.
	Past medical history+	
	Past medical history Have you been diagnosed with any medical problems in the past? For example, high blood pressure, diabetes?	I have high blood pressure.
	Allergies Any allergies to medications?	No.
	Allergies	

	Any allergies to certain foods?	No.
	Any allergies to latex?	No.
	Medications Do you take any medications?	No. I used to take something for my blood pressure, but the prescription ran out a few months ago, and I have not had time to pick up a refill.
	Medications Do you remember what medication it was?	I think it started with an "L" something.
	Hospitalizations Any overnight stays at the hospital?	Just for the birth of my two children a long time ago.
	Past surgical history Any surgeries in the past?	None.
	Family history Any health problems in the family? How are your parents doing? How's their health?	My mother has migraines, and my father also has high blood pressure.
	Social history	
	Do you smoke tobacco?	No.
	Have you ever smoked tobacco in the past?	Yes.
	How much did you smoke before you quit?	I smoked about 2 packs every day for about 20 years. I quit when I turned 50.
	How much alcohol do you drink?	I haven't drunk any alcohol for the past year.
	Recreational drugs Anything else? Marijuana, cocaine, methamphetamines, etc.	Never.
	Diet	I would say we have a balanced diet. We eat a lot of vegetables from our own garden.
	Caffeine	I start every workday with a cup of black coffee.
	Exercise	I walk the dog every day for about 20 minutes.
	Work	I work part-time as a front desk receptionist for a clinic in Vacaville.
	Home What is your living situation like? Who lives at home with you?	I live with my husband and our dog.
	Sexual Activity	Yes, with my husband only.
	EMPATHY QUESTION ASK BEFORE OR DURING THE PHYSICAL	I asked my son to bring me here, so can I call my husband and tell him I'm here with the doctor now? I am sure he is worried sick!

	Physical Exam	
	Washed hands	Student Dr. must wash hands before examining the patient, or else it will count as a Red Flag.
	Handled patient's gown respectfully Used proper draping as needed	Student Dr. should auscultate/palpate directly on skin when they can, unless it's over a sensitive region of the body.
	General Assessment <i>Empathy opportunity/Sample Response:</i> "Mrs. Carol, I understand that you would like to let your husband know where you are. Feel free to give him a call. If you cannot get ahold of him, I can also ask one of our medical assistants to let your son come into the room, if you're okay with that. How does that sound?"	CARD #1 55-year-old woman in severe/acute distress secondary to pain. Is lying very still on the table because movement elicits pain.
	Head, Eyes, Ears, Nose, Throat (HEENT)	
	Inspection of the head	Normocephalic, atraumatic.
	Palpation of the head	Non-tender.
	Inspection of the conjunctivae	No conjunctival injection or lacrimation.
	Direct fundoscopy	Normal.
	Neck	
	Inspection of the neck	Normal.
	Palpation of the neck for lymphadenopathy	No lymphadenopathy.
	Focused Osteopathic Structural Exam	CARD #2
	Prioritize thoracic region, T1-T4 (sympathetics to the head and neck). - Tissue texture changes - Tenderness	T1-T4 region: Increased temperature, increased skin drag, increased moisture, boggy, and tender. Hypertonic paraspinal muscles.
	Cardiovascular	
	Auscultation of the heart ("APTM")	Tachycardic. Regular rhythm. S1 S2 present. No murmurs.
	Respiratory	
	Auscultation of the posterior lung fields	Clear to auscultation bilaterally. No crackles or wheezes.
	Abdomen	
	Inspection of the abdomen	Soft, non-distended.
	Auscultation of the abdomen	Bowel sounds present and normoactive in all 4 quadrants.
	Light and deep palpation of the abdomen	Non-tender.

	Neurologic	
	<u>Mental Status</u> Orientation to person, place, and time. Person (already done when asking the patient how they like to be addressed) Place: Can you tell me where you are? Time: What's today's date?	Place: Touro Clinic in Vallejo Time: Ohh, I'm not sure... Wednesday, October 21, 2022?
	<u>Cranial Nerves</u> - Visual Fields (CN II) - EOM (CNs III, IV, and VI; can be done with HEENT) - Facial Sensation and Strength of the muscles of mastication (CN V; can be done with HEENT) - Facial movements (CN VII) - Hearing with finger rub test or whisper test (CN VIII) - Elevation of the soft palate and uvula (CN IX and X) - Strength of trapezius & SCM (CN XI) - Movement of the tongue (CN XII)	Cranial nerves II-XII all intact bilaterally. Active movement of the neck is slow and limited due to significant pain (when assessing CN XI).
	<u>Motor</u> / ROM of the cervical spine Brudzinski sign Kernig sign	<u>CARD #3</u> Active movement of the neck is restricted due to significant pain. Flexion is painful. Passive flexion of the neck results in spontaneous flexion of the hips. Passive extension at the knee elicits pain and muscle guarding against extension.
	<u>Coordination</u> - Rapid alternating movement test - Finger-Nose-Finger Test	Normal.
	<u>Reflexes, Deep Tendon</u> - Biceps (C5-C6), Brachioradialis (C5-C6), Triceps (C7-C8), Patellar (L2/L3-L4), Achilles (S1-S2)	DTRs 2+/4 bilaterally and equal.
	<u>Sensation</u>	Normal.
	<u>Gait</u>	Normal.
	Babinski sign	Plantarflexion of all toes bilaterally.
	Romberg test	Closing the eyes does not affect the patient's balance.

Adapted from AMBOSS's Clinical Skills Case 33.